This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

Wegovy (semaglutide) Injection

for weight management

Patient Group Direction, version 011, issued 30 September 2026. Supersedes Version 010, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE guidance and the SmPC for Wegovy (semaglutide) injection for weight management in adults

NICE technology appraisal guidance TA875, Semaglutide for managing overweight and obesity, published 8 March 2023, last updated 4 September 2023 (recommendations chapter read). Wegovy 2.4 mg FlexTouch solution for injection in pre-filled pen, Summary of Product Characteristics, Novo Nordisk Limited, last updated on emc 1 September 2026 (medicines.org.uk product 13803), sections 1 to 4.9 and 5.1 read. Summarised 10 September 2026.

NICE eligibility (TA875 recommendation 1.1)

Semaglutide is recommended as an option for weight management, including weight loss and weight maintenance, alongside a reduced-calorie diet and increased physical activity in adults, only if all of the conditions in recommendation 1.1 are met.

It must be used for a maximum of 2 years, and within a specialist weight management service providing multidisciplinary management of overweight or obesity (including but not limited to tiers 3 and 4).

The person must have at least 1 weight-related comorbidity and either a body mass index (BMI) of at least 35.0 kg/m2, or a BMI of 30.0 kg/m2 to 34.9 kg/m2 and meet the criteria for referral to specialist overweight and obesity management services in NICE's guideline on overweight and obesity management.

The company must provide semaglutide according to the commercial arrangement.

Use lower BMI thresholds (usually reduced by 2.5 kg/m2) for people from South Asian, Chinese, other Asian, Middle Eastern, Black African or African-Caribbean family backgrounds.

NICE explains that people from some minority ethnic family backgrounds have an equivalent risk from obesity at a lower BMI than people from a White ethnic family background, so a similar adjustment in the BMI threshold is appropriate when considering semaglutide.

NICE states that there is no evidence of effectiveness if semaglutide is used as a single stand-alone treatment, and that the marketing authorisation specifies use as an adjunct to a reduced-calorie diet and increased physical activity, which is why use is tied to lifestyle interventions provided in specialist weight management services.

NICE treatment duration and stopping rule (TA875 recommendations 1.2 and 1.3)

Treatment is limited to a maximum of 2 years.

Consider stopping semaglutide if less than 5 percent of the initial weight has been lost after 6 months of treatment.

The recommendations are not intended to affect treatment with semaglutide that was started in the NHS before the guidance was published; people having treatment outside the recommendations may continue without change to their existing funding arrangements until they and their NHS clinician consider it appropriate to stop.

Licensed indication for weight management (SmPC section 4.1)

Wegovy is indicated as an adjunct to a reduced-calorie diet and increased physical activity for weight management, including weight loss and weight maintenance, in adults with an initial BMI of 30 kg/m2 or more (obesity), or 27 kg/m2 to less than 30 kg/m2 (overweight) in the presence of at least one weight-related comorbidity.

The SmPC also carries indications for adolescents aged 12 years and above (obesity on CDC growth charts and body weight above 60 kg), for cardiovascular risk reduction in adults with established cardiovascular disease and BMI of 27 kg/m2 or more, and for non-cirrhotic metabolic dysfunction-associated steatohepatitis (MASH) with F2 to F3 fibrosis; these indications are outside the scope of adult weight management and are not summarised further here.

Dose escalation schedule (SmPC section 4.2, Table 2)

The maintenance dose of 2.4 mg once weekly is reached by starting at 0.25 mg; to reduce the likelihood of gastrointestinal symptoms the dose is escalated over a 16-week period.

Weeks 1 to 4: 0.25 mg once weekly. Weeks 5 to 8: 0.5 mg once weekly. Weeks 9 to 12: 1 mg once weekly. Weeks 13 to 16: 1.7 mg once weekly. Maintenance dose (all indications): 2.4 mg once weekly.

If needed, for weight management in patients with obesity, the dose can be increased to 7.2 mg once weekly after a minimum of 4 weeks on the 2.4 mg dose; the SmPC lists 7.2 mg as a maintenance dose for weight management in adult patients with obesity, if needed.

In case of significant gastrointestinal symptoms, consider delaying dose escalation or lowering to the previous dose until symptoms have improved.

For adolescents the same escalation applies up to 2.4 mg or the maximum tolerated dose; weekly doses higher than 2.4 mg are not recommended in adolescents.

The 5 percent rule, missed doses and switching (SmPC section 4.2)

If patients have been unable to lose at least 5 percent of their initial body weight after 6 months on treatment, a decision is required on whether to continue treatment, taking into account the benefit and risk profile in the individual patient.

If a dose is missed it should be administered as soon as possible and within 5 days after the missed dose; if more than 5 days have passed the missed dose should be skipped and the next dose given on the regularly scheduled day. If more doses are missed, reducing the starting dose for re-initiation should be considered.
Patients treated with semaglutide 25 mg tablets once daily can be transitioned to semaglutide injection 2.4 mg once weekly, starting the injection the day after the last tablet dose; the SmPC notes that the effect of switching cannot easily be predicted because oral semaglutide has higher pharmacokinetic variability in absorption.
Semaglutide should not be used in combination with other GLP-1 receptor agonist products. When initiating semaglutide in a patient with type 2 diabetes, consider reducing the dose of concomitant insulin or insulin secretagogues (such as sulfonylureas) to reduce the risk of hypoglycaemia.

Special populations (SmPC section 4.2)

No dose adjustment is required based on age; therapeutic experience in patients aged 85 years or over is limited.
No dose adjustment is required for mild, moderate or severe renal impairment; experience in severe renal impairment is limited, and semaglutide is not recommended in end-stage renal disease.
No dose adjustment is required for hepatic impairment (Child-Pugh A, B or C); experience in severe hepatic impairment (Child-Pugh C) is limited and caution should be exercised.
The safety and efficacy of semaglutide in children below 12 years of age have not been established.

Method of administration and presentation (SmPC sections 2 and 4.2)

Wegovy is administered once weekly at any time of the day, with or without meals, injected subcutaneously in the abdomen, thigh or upper arm; the injection site can be changed. It must not be given intravenously or intramuscularly.
The day of weekly administration can be changed if necessary as long as the time between doses is at least 3 days (more than 72 hours); once-weekly dosing then continues from the new day.
The Wegovy 2.4 mg FlexTouch pen contains 3.2 mg/mL of semaglutide; each dose contains 2.4 mg in 0.75 mL, and one pre-filled pen contains 9.6 mg (four doses) in 3 mL.
For the 7.2 mg dose, administer either 1 injection of 7.2 mg or 3 injections of 2.4 mg one after each other, depending on the device; the three injections can be given in the same body area but at least 5 cm apart, doses from more than one pen may be needed, and the needle should be changed between each dose.
The emc product listing for Wegovy on 10 September 2026 shows FlexTouch pens in 0.25 mg, 0.5 mg, 1 mg, 1.7 mg and 2.4 mg strengths and a Wegovy 7.2 mg solution for injection in pre-filled pen, alongside the 1.5 mg, 4 mg, 9 mg and 25 mg tablets.
Patients should be advised to read the instructions for use in the package leaflet carefully before administering the product.

Contraindications (SmPC section 4.3)

Hypersensitivity to the active substance or to any of the excipients listed in section 6.1. No other contraindications are listed.

Warnings and precautions (SmPC section 4.4)

Gastrointestinal effects and dehydration: nausea, vomiting and diarrhoea may cause dehydration, which in rare cases can lead to a deterioration of renal function; this should be considered in patients with impaired renal function, and patients should be advised of the risk and told to take precautions to avoid fluid depletion.
Aspiration with general anaesthesia or deep sedation: cases of pulmonary aspiration have been reported in patients on GLP-1 receptor agonists; the increased risk of residual gastric content due to delayed gastric emptying should be considered before procedures with general anaesthesia or deep sedation.
Acute pancreatitis: semaglutide has not been studied in patients with a history of pancreatitis and should be used with caution in them. Acute pancreatitis, including post-marketing reports of necrotising pancreatitis and fatal outcomes, has been reported with GLP-1 receptor agonists. Patients should be told the symptoms (persistent, severe abdominal pain) and to seek immediate medical attention; if pancreatitis is suspected semaglutide should be discontinued, and if confirmed it should not be restarted. Raised pancreatic enzymes alone are not predictive of acute pancreatitis.
Non-arteritic anterior ischaemic optic neuropathy (NAION): epidemiological data may indicate an increased risk during semaglutide treatment with no identified time interval; patients reporting sudden loss of vision (including partial loss) should be urgently referred for ophthalmological examination, and semaglutide discontinued if NAION is confirmed.
Semaglutide must not be used as a substitute for insulin in patients with diabetes.
Hypoglycaemia: semaglutide lowers blood glucose and can cause hypoglycaemia; patients should be educated on the signs and symptoms. Patients treated with semaglutide together with a sulfonylurea or insulin may have an increased risk, which can be lowered by reducing the sulfonylurea or insulin dose when starting a GLP-1 receptor agonist.

Diabetic retinopathy: in patients with diabetic retinopathy treated with insulin and semaglutide, an increased risk of diabetic retinopathy complications has been observed; such patients should be monitored closely and treated according to clinical guidelines. There is no experience with semaglutide 2.4 mg in type 2 diabetes with uncontrolled or potentially unstable diabetic retinopathy.
Gastroparesis: patients with gastroparesis may experience more serious or severe gastrointestinal adverse events; use with caution, and semaglutide is not recommended if gastroparesis is severe.
Populations not studied: there is no experience in congestive heart failure NYHA class IV and limited experience in patients aged 85 years or more.
Traceability: the name and batch number of the administered product should be clearly recorded. The product contains less than 1 mmol sodium per dose.

Interactions (SmPC section 4.5)

Semaglutide may delay gastric emptying and could potentially influence the absorption of concomitant oral medicines, but no clinically relevant effect on the rate of gastric emptying was observed with semaglutide 2.4 mg and no clinically relevant drug-drug interactions were seen in clinical pharmacology trials, so no dose adjustment is required for co-administered medicines.
Co-administration with other semaglutide-containing products or with any other GLP-1 receptor agonist is not recommended.
Oral contraceptives: semaglutide is not anticipated to decrease the effectiveness of oral contraceptives; ethinylestradiol exposure was unaffected and levonorgestrel exposure increased by 20 percent at steady state, with no change in Cmax.
Atorvastatin, digoxin and metformin: overall exposure was not changed (atorvastatin Cmax was decreased by 38 percent, assessed as not clinically relevant).
Warfarin and other coumarin derivatives: overall exposure and INR were not affected in a clinically relevant manner, but cases of decreased INR have been reported with acenocoumarol, and frequent INR monitoring is recommended when semaglutide is started in patients on warfarin or other coumarins.

Pregnancy, breastfeeding and contraception (SmPC section 4.6)

Women of childbearing potential are recommended to use contraception when treated with semaglutide.
Semaglutide should not be used during pregnancy; animal studies have shown reproductive toxicity and there are limited data in pregnant women. If a patient wishes to become pregnant, or pregnancy occurs, semaglutide should be discontinued.
Semaglutide should be discontinued at least 2 months before a planned pregnancy because of its long half-life.
Semaglutide should not be used during breast-feeding; a risk to a breast-fed child cannot be excluded.
The effect on human fertility is unknown.

Driving (SmPC section 4.7)

Semaglutide has no or negligible influence on the ability to drive or use machines, but dizziness can occur, mainly during dose escalation, and driving or machine use should be done cautiously if it does. Patients on a sulfonylurea or insulin should take precautions to avoid hypoglycaemia while driving.

Adverse effects (SmPC section 4.8)

The most frequently reported adverse reactions are gastrointestinal: nausea, diarrhoea, constipation and vomiting.
Very common (1 in 10 or more): headache, vomiting, diarrhoea, constipation, nausea, abdominal pain, fatigue.
Common (1 in 100 to 1 in 10): hypoglycaemia in patients with type 2 diabetes, dizziness, dysaesthesia, dysgeusia, diabetic retinopathy in type 2 diabetes, gastritis, gastro-oesophageal reflux disease, dyspepsia, eructation, flatulence, abdominal distension, cholelithiasis, hair loss, injection site reactions.
Uncommon (1 in 1,000 to 1 in 100): hypoglycaemia in patients without type 2 diabetes, increased heart rate, hypotension, orthostatic hypotension, acute pancreatitis, delayed gastric emptying, urolithiasis, increased amylase, increased lipase, increased bilirubin.
Rare: anaphylactic reaction, angioedema, hip fracture. Very rare: NAION. Frequency not known: intestinal obstruction (post-marketing).
Over 68 weeks in the phase 3a trials, nausea occurred in 43.9 percent of patients on 2.4 mg (16.1 percent placebo), diarrhoea in 29.7 percent (15.9 percent), vomiting in 24.5 percent (6.3 percent) and constipation in 24.2 percent (11.1 percent); gastrointestinal events led to permanent discontinuation in 4.3 percent.
Adjudication-confirmed acute pancreatitis occurred in 0.2 percent on semaglutide 2.4 mg; cholelithiasis in 1.6 percent, leading to cholecystitis in 0.6 percent; hair loss in 2.5 percent (1.0 percent placebo), more often with greater weight loss; mean heart rate rose by 3 beats per minute.
With the 7.2 mg dose in the STEP UP trials, dysaesthesia was reported in 21.6 percent (0.3 percent placebo) and hair loss in 5.3 percent.

Large epidemiological studies suggest semaglutide exposure in adults with type 2 diabetes may be associated with an approximately two-fold increase in the relative risk of NAION, corresponding to approximately one additional case per 10,000 person-years of treatment.
Overdose may be associated with gastrointestinal disorders that could lead to dehydration; observe and give supportive treatment.
Wegovy is subject to additional monitoring (black triangle); suspected adverse reactions should be reported via the MHRA Yellow Card scheme.

What the sources do not say

TA875 does not refer to the 7.2 mg dose, to community pharmacy, or to private supply; its recommendation is for use within a specialist weight management service.
The Wegovy SmPC section 4.4 has no separate gallbladder disease warning; gallbladder events (cholelithiasis, cholecystitis) appear only as adverse reactions in section 4.8.
The Wegovy SmPC does not mention suicidal ideation or behaviour.
The SmPC text retrieved ended within section 5, so storage, shelf life and pen handling (section 6) are not summarised here.

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must be competent in weight management consultation, BMI calculation and the recognition of red flags requiring referral • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005

Initial Assessment

Before commencing treatment with weight management therapies, the patient must be assessed by a registered healthcare professional. The initial assessment should be face to face. The assessment should include, but may not be limited to:

- Causes of weight gain. Refer the patient to their GP if prescribed medication is causing weight gain.
- The patient’s lifestyle, diet and level of exercise.
- Has the patient tried to lose weight in the past? Discuss what has and has not worked.
- Other factors that may be causing weight gain, including mental health, environmental and psychological factors. Consider signposting to another health professional, for example a mental health team.
- Other disease states that may predispose to weight gain. Consider referring to the GP for further advice.

- The patient's expectations of weight loss, and whether they are realistic for their height and body frame.
- Calculate BMI and determine the ideal weight. Set a realistic target weight and agree review intervals.

Clinical condition or situation to which this PGD applies

PGD Indication	Wegovy (semaglutide) is a GLP-1 receptor agonist indicated as an adjunct to a reduced-calorie diet and increased physical activity for weight management, including weight loss and weight maintenance, in adults with an initial Body Mass Index (BMI) of 30 kg/m² or above (obesity), or 27 kg/m² to below 30 kg/m² (overweight) in the presence of at least one weight-related comorbidity, for example hypertension, dyslipidaemia, obstructive sleep apnoea, cardiovascular disease or type 2 diabetes.
Inclusion criteria	• Adults aged 18 to 75 years (inclusive). • BMI 30 kg/m² or above, or BMI 27 kg/m² or above with at least one weight-related comorbidity (for example hypertension, type 2 diabetes mellitus, dyslipidaemia, obstructive sleep apnoea, established cardiovascular disease). • Patient is willing to follow a reduced-calorie diet and increase physical activity in line with the agreed lifestyle plan. • Initial assessment completed and documented (see the Initial Assessment section above). • Informed consent obtained, including off-label considerations where applicable, the side-effect profile and treatment expectations. • No exclusion criteria present. • Progression to the 7.2 mg dose is permitted only where the patient's STARTING BMI was 30 kg/m² or above. Patients who started at a BMI of 27 to below 30 kg/m² are restricted by the manufacturer to a maximum dose of 2.4 mg once weekly.
Exclusion criteria	• Patients under 18 years of age. • Adults aged over 75 years (upper age limit under this PGD; refer to a specialist if treatment is being considered). • History of suicidal ideation or active severe mental illness where psychiatric oversight is absent and there is current concern about mood or risk (see cautions). • Pregnancy, breastfeeding, or planning pregnancy. Effective contraception is required; advise discontinuation at least 2 months before a planned conception (SmPC). • Known hypersensitivity to semaglutide or to any of the excipients. • Personal or family history of medullary thyroid carcinoma (MTC) or Multiple Endocrine Neoplasia syndrome type 2 (MEN 2). This exclusion, like pancreatitis, is stricter than the UK SmPC, which lists no contraindication other than hypersensitivity; it follows the US labelling and is a deliberate service restriction. • History of pancreatitis, acute or chronic. • Severe gastrointestinal disease, including gastroparesis or severe persistent gastrointestinal disorder. • Current cholelithiasis (gallstones) or cholecystitis, or cholecystectomy within the last 3 months. • Concurrent use of any other GLP-1 receptor agonist or insulin

	secretagogue, FOR ANY INDICATION. Ask specifically about medicines taken for diabetes and name the products: oral or injectable semaglutide, tirzepatide, orforglipron, liraglutide, dulaglutide, exenatide, and the sulfonylureas and meglitinides. Patients do not always think of a diabetes medicine as the same kind of drug as a weight loss one. ● Type 1 diabetes mellitus. ● Diabetic retinopathy. Treatment may worsen retinopathy; defer or refer to a specialist. ● Insulin-treated diabetes. Refer: a pharmacy weight-management service cannot manage insulin dose reduction. ● Severe renal impairment (eGFR below 30 mL/min/1.73 m²) or end-stage renal disease. ● Severe hepatic impairment. ● Known diagnosis of heart failure with reduced ejection fraction below 40%. ● Active eating disorder (anorexia nervosa, bulimia, or binge-eating disorder under specialist care). ● BMI below the PGD inclusion threshold. ● Patients who, in the clinical judgement of the healthcare professional, are not suitable for the medication.
Cautions	● Counsel on gradual dose escalation and gastrointestinal side effects; consider delaying titration or reducing to the previous dose if significant symptoms occur. ● History of suicidal ideation, or active severe mental illness: supply only where appropriate psychiatric oversight is in place; monitor mood at review and refer if there is any concern. Where oversight is absent and concern exists, the exclusion criteria apply. ● Non-arteritic anterior ischaemic optic neuropathy (NAION) has been reported with semaglutide. Advise patients to seek urgent review for any sudden loss of vision, and discontinue if NAION is confirmed. ● Mild to moderate renal impairment: monitor for dehydration secondary to gastrointestinal side effects. ● An endocrine cause of obesity is not an exclusion under this PGD: a patient with treated hypothyroidism or treated Cushing's syndrome may be supplied. Where an endocrine cause is suspected and has not been assessed or treated, inform the GP so that it can be investigated and record this in the notes; this is a caution to inform the GP, not a reason to withhold supply. ● For individuals who take HRT, due to the lack of data regarding absorption, non-oral products (for example patch, gel, or levonorgestrel intrauterine device) may be considered. ● Semaglutide delays gastric emptying and thereby has the potential to impact the rate of absorption of concomitantly administered oral medicinal products, especially those with a narrow therapeutic index. Upon initiation of semaglutide treatment in patients on warfarin, frequent monitoring of INR is recommended. ● Cases of pulmonary aspiration have been reported in patients receiving GLP-1 receptor agonists undergoing general anaesthesia or deep sedation for surgery. The increased risk of residual gastric content due to delayed gastric emptying should

	therefore be considered before procedures under general anaesthesia or deep sedation. ● Cases of tachycardia have been reported. In patients with a pre-existing increased heart rate, use with caution and seek specialist advice before use. If a patient experiences a clinically relevant sustained increase in resting heart rate, treatment should be discontinued and advice sought. ● Counsel on the signs and symptoms of pancreatitis; discontinue and refer urgently if suspected. ● Type 2 diabetes on metformin, an SGLT2 inhibitor or a DPP-4 inhibitor only: no dose adjustment is needed, but inform the GP. Any sulfonylurea, meglitinide or insulin EXCLUDES (see exclusion criteria); there is no GP-monitored route for those patients under this PGD. ● Risk of dehydration. Counsel on adequate fluid intake during gastrointestinal side effects. ● Treatment under this PGD is stopped where less than 5% of initial body weight has been lost after 6 months on the maximum tolerated dose; refer to the GP or a specialist prescriber for a decision on any continuation (see maximum treatment period). ● Rotate injection sites to reduce local irritation.
Actions if patient is excluded or declines treatment	● Discuss the reason for exclusion with the patient and ensure they understand. ● Advise on alternative treatment options and how these can be accessed, for example the GP, a specialist weight management service, or lifestyle programmes. ● If the exclusion relates to an undiagnosed or unmanaged comorbidity, recommend GP review. ● Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	● Wegovy FlexTouch solution for injection in pre-filled pen. One pre-filled pen contains 4 doses of semaglutide. Each dose contains 0.25 mg, 0.5 mg, 1.0 mg, 1.7 mg or 2.4 mg of semaglutide. ● Wegovy 7.2 mg solution for injection in pre-filled pen. Four pre-filled single use pens; each pen contains 7.2 mg. The pen should be pressed firmly against the skin until the yellow bar has stopped moving. The injection takes about 5 to 10 seconds.
Legal category	POM
Storage	● Store in a refrigerator (2°C to 8°C). Do not freeze; discard if frozen. ● After first use, the FlexTouch pen can be stored for up to 6 weeks below 30°C or in a refrigerator (2°C to 8°C). ● Keep the pen cap on when the pen is not in use in order to protect it from light. ● Travel: carry pens in hand luggage when travelling by air. Pens should not be placed in checked baggage where temperatures cannot be guaranteed. A travel letter from the prescriber or pharmacist may be required.
Route/method of administration	● Administered once weekly at any time of day, with or without

	meals. ● Injected subcutaneously in the abdomen, thigh or upper arm. The injection site can be changed. Do not administer intravenously or intramuscularly. ● For the 7.2 mg dose, use the Wegovy 7.2 mg single use pen and follow the instructions for that presentation. The 7.2 mg dose is not made up from three 2.4 mg FlexTouch injections under this PGD. ● The day of weekly administration can be changed if necessary as long as the time between weekly doses is at least 3 days (more than 72 hours). After selecting a new dosing day, once-weekly dosing should be continued. ● Patients should read the instructions for use in the package leaflet carefully before administering.
Dose and frequency	● The maintenance dose of 2.4 mg once weekly is reached by starting at 0.25 mg and escalating over 16 weeks: weeks 1 to 4, 0.25 mg; weeks 5 to 8, 0.5 mg; weeks 9 to 12, 1.0 mg; weeks 13 to 16, 1.7 mg; week 17 onwards, 2.4 mg once weekly. ● If needed for weight management in patients with obesity, the dose may be increased to 7.2 mg once weekly after a minimum of 4 weeks on 2.4 mg. This is permitted only where the starting BMI was 30 kg/m² or above; patients who started between 27 and below 30 kg/m² remain at a maximum of 2.4 mg. ● In the case of significant gastrointestinal symptoms, consider delaying dose escalation or lowering to the previous dose until symptoms have improved. ● If a dose is missed, administer as soon as possible and within 5 days of the missed dose. If more than 5 days have passed, skip the missed dose and administer the next dose on the regularly scheduled day, then resume the usual weekly schedule. ● If more than 2 doses are missed, reduce and re-escalate the dose to avoid unwanted side effects when treatment is re-initiated.
Quantity to be administered and/or supplied	● All FlexTouch strengths contain 4 doses with 4 disposable needles, giving one month of treatment. ● The 7.2 mg presentation is supplied as four pre-filled single use pens, giving one month of treatment. ● Supply one month of treatment per patient appointment. ● This PGD does not allow additional medication to be supplied to enable patients to stock up. ● Maximum treatment period: 2 years of continuous treatment under this PGD, in line with NICE TA875, after which the patient is referred to the GP or a specialist prescriber for a decision on continuation. Treatment is stopped earlier where less than 5% of initial body weight has been lost after 6 months on the maximum tolerated dose, or where the patient no longer meets the criteria. ● NICE TA875 recommends semaglutide within a specialist weight management service with multidisciplinary support. This PGD authorises private supply outside that commissioning position: the pharmacist provides the diet, activity and behavioural support described under counselling, the GP is informed at initiation and at each review, and the patient is told that the NHS route exists and how to access it.
Monitoring and review	● Reassess clinical benefit, tolerability and target weight at each

	visit. ● If the patient has not lost at least 5% of their initial body weight after 6 months on the maximum tolerated dose, treatment under this PGD is stopped and the patient is referred to the GP or a specialist prescriber for a decision on any continuation. Record the weights, the percentage lost and the decision. ● When the patient reaches their target weight, discuss whether to continue treatment to maintain it. ● If the patient has used Wegovy in the past and wants to recommence treatment, the dose must be titrated again from the lowest dose. The BMI inclusion criteria for initiation must be reapplied if more than 2 months have passed since discontinuing treatment.
Adverse effects	Very common: nausea, diarrhoea, vomiting, constipation, abdominal pain, headache and fatigue. Common: dyspepsia, eructation, flatulence, gastro-oesophageal reflux, gallstones, dizziness, hypoglycaemia when used with a sulfonylurea or insulin, injection site reactions, hair loss and tachycardia. Uncommon or rare: acute pancreatitis, cholecystitis, hypersensitivity reactions including anaphylaxis, acute kidney injury usually secondary to dehydration, and non-arteritic anterior ischaemic optic neuropathy. Refer to the current SmPC for full safety information.
Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP with whom the individual is registered ● name of healthcare practitioner ● name and brand of medication, and batch number ● date of supply, dose, form, route and quantity supplied ● whether the supply is a start, a restart, a dose escalation, a dose reduction or a continuation, and the previous dose ● height, weight and BMI at this visit, and the target weight agreed ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken ● that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. Adults aged 18 and over: keep records for 8 years.

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medicine, together with written lifestyle, diet and physical activity advice and the agreed target weight.
Follow-up advice to be given to patient or carer	Explain the expected pattern of weight loss and that the medicine works alongside diet and activity, not instead of them. Counsel on gastrointestinal side effects and how to manage them, on adequate fluid intake, and on the warning symptoms that need urgent attention: severe abdominal pain, persistent vomiting with dehydration, jaundice, sudden visual loss, or a sustained rise in resting heart rate. Advise the patient that review is at each monthly supply, and that treatment under this PGD will

	stop if less than 5% of body weight has been lost after 6 months on the maximum tolerated dose.

Key references

- Summary of Product Characteristics for the product supplied, current version
- NICE guidance on obesity management and on the relevant medicine
- MHRA Drug Safety Updates relevant to GLP-1 receptor agonists
- British National Formulary (BNF)
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates	

Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date
30/9/26	31/7/27

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

To be completed by the adopting pharmacy: the first block is signed by that pharmacy's superintendent or clinical lead, and the second by its professional group signatory. Get Real Health signs the authorship block below, not these.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 002

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development and issue of new PGD	1st May 2026
002	PPH clinical review (J. Wilkins with S. Passmore). Title changed to Wegovy (semaglutide) Injection because the 7.2 mg pen is not a FlexTouch presentation, and 7.2 mg separated out in the medicine details with its own administration instructions. Progression to 7.2 mg restricted to patients whose starting BMI was 30 kg/m² or above, in line with the manufacturer. Adjusted BMI thresholds for ethnic groups removed from the inclusion criteria. Gallbladder disease moved from cautions to exclusions. Obesity caused by an endocrinological disorder added as an exclusion. History of suicidal ideation or active severe mental illness moved from exclusions to cautions. NAION caution added. Oral contraceptive wording removed from the concomitant medicines caution as it is not required for semaglutide. Cautions added covering HRT absorption, delayed gastric emptying with INR monitoring on warfarin, pulmonary aspiration under general anaesthesia or deep sedation, and tachycardia. Dosing now states that if more than 2 doses are missed the dose is reduced and re-escalated. Inclusions, exclusions and cautions aligned with the Wegovy tablet PGD.	6th August 2026

Version and change record
Version: v011
Issued: 30 September 2026
Supersedes: Version 010, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

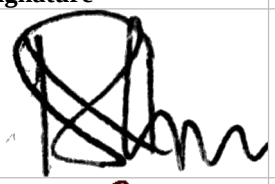
Previous version records
Version: v010
Issued: 14 September 2026
Supersedes: Version 009, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v009
Issued: 11 September 2026
Supersedes: Version 008, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Decision 48: the exclusion for obesity caused by an endocrinological disorder is removed. An endocrine cause of obesity does not exclude; treated hypothyroidism and treated Cushing's syndrome may be supplied. A caution now says that where an endocrine cause is suspected and has not been assessed or treated the GP is informed so that it can be investigated.
Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.
Previous version records
Version: v008
Issued: 11 September 2026
Supersedes: Version 007, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Exclusion wording corrected: patients under 18, not adults under 18.
Pregnancy exclusion no longer mentions tirzepatide, which is not a product under this PGD.
The medullary thyroid carcinoma and MEN 2 exclusion states its basis (US labelling; stricter than the UK SmPC).
The mental health rule that ended the cautions row (do not supply where oversight is absent and concern exists) is now an exclusion criterion, with the caution pointing to it.
Duplicate delayed gastric emptying caution removed.
The 5% rule is stated one way in the cautions, monitoring and follow-up rows: treatment under this PGD stops after 6 months on the maximum tolerated dose without 5% loss, with referral, as the maximum treatment period row and the version 002 change record already said.

The 7.2 mg dose is given with the 7.2 mg single use pen only; the route row no longer permits three 2.4 mg injections, which did not fit the one pen per month quantity row.
Records include the nature of the supply (start, restart, escalation, reduction, continuation) and the previous dose.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v007
Issued: 11 September 2026
Supersedes: Version 006, 10 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
A maximum treatment period row added: 2 years under this PGD (NICE TA875), stop at 6 months on the maximum tolerated dose if under 5% weight loss. The document now states that supply is private and outside the NICE commissioning position, and what the pharmacy provides in place of the specialist service.
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.
Previous version records
Version: v003
Issued: 9 September 2026
Supersedes: the previous signed version of this document, and any correction notice attached to it.
Change: The concurrent GLP-1 exclusion read "used for weight management". Several GLP-1 medicines are licensed for type 2 diabetes as well, so a patient already taking one for diabetes was not caught and could have been supplied a second. The exclusion now reads "for any indication" and names the products to ask about. This incorporates and withdraws the correction notice of 7 September 2026. No other change.
Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.
Version: v004
Issued: 10 September 2026
Supersedes: Version 003, 9 September 2026
Change: the summary of the governing guidance is added as part 2 of the document, between the cover and the PGD, which is the house format for every Get Real Health PGD. It is summarised from the source it names and adds nothing to it. Nothing else in this document is altered, and this is not a clinical review of the remainder.
Version: v005
Issued: 10 September 2026
Supersedes: Version 004, 10 September 2026
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. No period of validity was stated.
Version: v006
Issued: 10 September 2026
Supersedes: Version 005, 10 September 2026
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:

One rule for insulin secretagogues and insulin. The previous version excluded every sulfonylurea or meglitinide user in one bullet, then excluded them only 'where the GP is unwilling to monitor' two bullets later, then made co-use a referral caution on the next page. The blanket exclusion stands; insulin-treated diabetes is now an exclusion in the same terms; the conditional exclusion and the caution are gone.

Signed on behalf of Get Real Health

Version v011, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.